

Pampara Injection 500mg/20ml

Pralidoxime chloride injection 500mg

Description

A clear and colorless to slightly yellow liquid in glass ampoules.

Composition

Each 20ml ampoule contains 500mg of Pralidoxime (as chloride).

Mode of Action

Pralidoxime is cholinesterase reactivator and its principal mode of action is by reactivating the enzyme cholinesterase which has been inhibited by phosphorylation.

Summary of Pharmacodynamics and Pharmacokinetics

Blood concentrations are rapidly attained after intramuscular or intravenous injection. Pralidoxime is not bound to plasma protein and does not readily pass into the central nervous system. Pralidoxime is rapidly excreted in the urine partly unchanged and partly as a metabolite. The elimination half-life is approximately 1 to 2 hours.

Indications

Pampara Injection is indicated in the treatment of poisoning due to organophosphates pesticides or related compounds. Pralidoxime is an adjunct to atropine in the treatment of poisoning due to those pesticides and chemicals of the organophosphate class which have anticholinesterase activity.

Pampara Injection is indicated in the treatment of overdosage by anticholinesterase drugs used in the treatment of myasthenia gravis.

Side Effects/ Adverse reaction

The side effects include drowsiness, dizziness, and disturbances of vision, nausea, tachycardia, headaches, hyperventilation, and muscular weakness have been reported after the use of Pampara injection. However it is very difficult to differentiate the toxic effects produced by atropine or the organophosphate compounds from those of the drug. When atropine and Pampara injection are used together, the signs of atropinization may occur earlier than might be expected when atropine is used alone. This is especially true if the total dose of atropine has been large and the administration of Pampara injection has been delayed. Excitement and manic behavior immediately following recovery of consciousness have been reported in several cases. However, similar behavior has occurred in case of organophosphate poisoning that were not treated with Pampara injection.

Warning/Precautions

Pampara injection has been very well tolerated in most cases, but it must be remembered that the desperate condition of the organophosphate-poisoned patients will generally mask such minor signs and symptoms as have been noted in normal subjects.

Intravenous administration of Pampara Injection should be carried out slowly and preferably, by infusion, since certain side effect, such as tachycardia, laryngospasm, and muscle rigidity, have been attributed in a few cases to a too rapid rate of injection.

Pampara Injection is excreted in the urine, a decrease in renal function will result in increased blood levels of the drug thus the dosage of Pampara injection should be reduced in the presence of renal insufficiency.

Pampara Injection should be used with great caution in treating organophosphate overdosage in cases of myasthenia gravis since it may precipitate a myasthenic crisis.

Contraindication

Pampara injection is contraindicated in patients of known hypersensitivity to the active ingredient and other situations in which the potential risk clearly outweighs potential benefit.

Recommended Dose

Pralidoxime may be administered intravenously by slow intravenous injection of a 5% to 10% solution in water for injection over 5 to 10 minutes or alternatively by infusion in 100 ml of sodium chloride injection (0.9%) over a period of 15 to 30 minutes.

In the treatment of organophosphorous poisoning, atropine sulfate 2 mg should be given by subcutaneous or intramuscular injection and repeated at intervals of 5 to 60 minutes until the patient shows signs of atropine toxicity: Atropinisation should then be maintained throughout the course of pralidoxime treatment which may continue for 48 hours or more large amounts of atropine may be required concomitantly. 1 to 2g of Pralidoxime should be administered intravenously and repeated, if necessary according to the patient's condition: a maximum dose of 12g of 24 hours has been suggested.

In children, Pralidoxime 20 to 60mg per kg body weight may be given depending on the severity of poisoning and response to treatment. The dose of Pralidoxime may need to be reduced in patients with impaired renal function.

Anticholinesterase overdosage

As an antagonist to such anticholinesterases as neostigmine, pyridostigmine, and ambenonium, which are used in the treatment of myasthenia gravis, Pampara may be given in a dosage of 1 to 2g intravenously followed by increments of 250 mg every five minutes.

Use in Pregnancy and Lactation

Pregnancy category C. Well-controlled studies in humans have not been done. Animal reproduction studies have not been conducted with Pralidoxime. It is also not known whether Pralidoxime can cause fetal harm when administered to a pregnant woman or can affect reproduction capacity. Pralidoxime should only be given to pregnant woman only if the potential benefit outweighs the potential risk.

It is not known whether this drug is excreted in human milk. Because many drugs are excreted in human milk, caution should be exercised when Pralidoxime is administered to a nursing woman.

Interaction with Other Medicaments/Drug Interactions

The following precautions should be kept in mind in the treatment of anticholinesterases poisoning. Although these drugs do not interact directly with the action of Pampara injection, the action of barbiturates are potentiated by the anticholinesterases. Hence the use of such drugs i.e. morphine, theophylline, aminophylline, succinylcholine, reserpine and phenothiazine-type tranquillizers should be avoided in the treatment of convulsion in patients of organophosphate poisoning.

Pampara should be used with great caution in treating organophosphate overdosage in cases of myasthenia gravis since it may precipitate a myasthenic crisis.

Because pralidoxime is excreted in the urine, a decrease in renal function will result in increased blood levels of the drug. Thus the dosage of pralidoxime should be reduced in the presence of renal insufficiency.

Incompatibilities

Pralidoxime may be diluted with 5% to 10% solution in water for injection or alternatively by infusion in 100 ml of sodium chloride injection (0.9%).

Symptoms and Treatment of Overdose

The symptoms observed in normal subjects include dizziness, blurred vision, diplopia, headache, impaired accommodation, nausea, slight tachycardia. In therapy it has been difficult to differentiate side effects due to the drug from those due to the effects of the poison. Treatment of over dosage includes artificial respiration and other supportive therapy.

Presentation

This injection is available in 20ml glass ampoules. Five ampoules are packed into one folder paper box.

Storage Condition

Protect from light and store below 30°Celsius. Keep out of reach of children.

Shelf Life

The injections can be used within 48 month from the date of manufacture if kept as recommended.

Registration number

Pampara Injection 500mg/20ml – **MAL06061535AZ**

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